

UF HEALTH SHANDS CORE POLICY AND PROCEDURE

POLICY NUMBER: CP02.100

CATEGORY: Patient Care

REVIEW RESPONSIBILITY/DOCUMENT OWNER: Mgr., Transfusion Services

TITLE: Blood Transfusion and Patient Blood Management

POLICY: A blood transfusion is a therapy used only when the clinical benefits to the patient outweigh the potential risks, due to the potential for serious complications/adverse effects, the most important of these being acute hemolytic reactions and transfusion-transmitted infections. Stringent procedures must be followed to ensure that the correct blood is given and that any adverse reactions are dealt with promptly and efficiently.

Blood transfusions must be conducted according to procedures annexed to this Policy and must only be conducted by staff who are trained and competent in the procedures. This Policy is applicable to any organizational policy that relates to Blood and Blood Products.

This Policy applies to all areas of UF Health Shands, and all employees of UF Health Shands, including individuals employed by a third party, by external contractors, as voluntary workers, as students, as locums, or as agency staff.

PURPOSE: Ensure that patients receive correctly matched and medically indicated blood products and that any adverse reactions are dealt with promptly and efficiently.

APPROVED:

 10/19/2023 3:01 PM EDT
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Timothy E. Morey, M.D.
Chief Medical Officer

 Digitally signed by James J. Kelly, Jr. Date: 2023.10.17 07:48:58 -04'00'
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James J. Kelly, Jr.
Interim Chief Executive Officer

(continued):

CP02.100 (2 of 6)

DEFINITIONS:

- A. Blood Components - Units, pedi-packs or pooled units of: Red blood cells, Platelets, Plasma, Cryoprecipitate, Leukocytes and Whole Blood.
- B. Blood Substitute - Any hemoglobin-based oxygen carrier for use in humans as an oxygen delivering bridge in cases when human blood is not available or is not an option.
- C. Alternatives to Blood Transfusion - The administration of any of the following in place of blood transfusion:
 - 1. Iron (Fe) therapy;
 - 2. Erythropoietin (EPO) or erythropoietin stimulating agents;
 - 3. Hemo-dilution;
 - 4. Intraoperative closed cell salvage; and
 - 5. Derivatives of primary blood component (Albumin, Fibrinogen, Clotting factor concentrates, IVIg, Recombinant factors, Blood substitute).

RESPONSIBILITIES:

The Chief Executive Officer/Chief Medical Officer have overall responsibility for ensuring that there is a safe system for transfusion practice within the organization as well as ensuring that health care professionals and ancillary staff are informed of and follow UF Health Shands' policy.

The Transfusion Services Medical Director(s) has/have been delegated authority for transfusion practice within UF Health Shands.

The UF Health Shands Hospital Transfusion Committee has delegated responsibility, on behalf of the Medical Executive Committee and the Hospital Board of Directors, to oversee, develop and implement UF Health Shands policies and procedures related to blood storage, transport and transfusion. The Transfusion Committee will conduct ongoing evaluation of transfusion practices to insure proper utilization of blood and its components and shall:

- 1. Review transfusion practices for proper utilization of all blood products including crossmatch transfusion ratio per provider and service.
- 2. Design audit strategies to assess the utilization of blood products that include development of appropriate criteria, and review of individual cases.
- 3. Review transfusion statistics that indicate the use of blood and blood products, e.g., the amount of blood requested, the amount used, and the amount outdated.

(continued):

CP02.100 (3 of 6)

4. Review of actual or suspected transfusion reactions and suspected cases of transfusion transmitted infectious disease.
5. Analyze identified problems or issues, formulate appropriate recommendations on best practice, and provide continuing education programs.
6. Inform the Physician/Physician designee of these recommendations and of changes in prevailing scientific understanding of good transfusion practice.
7. Assure that blood products are readily available to satisfy each clinical need, provided in a prompt manner and at a reasonable cost.

The UF Health Shands Hospital Quality and Patient Safety Committee (QPSC) is responsible for:

1. Ensuring that policies on patient identification are in place, implemented and monitored throughout the blood transfusion process - from prescription, sampling, laboratory testing, and issuance of blood to administration of blood transfusion;
2. Ensuring that staff who are involved in the blood transfusion process are competent to follow these procedures;
3. Availability of written information to patients about the risks, benefits and potential alternatives to blood transfusion and that consent or refusal is documented;
4. Staff training and availability of equipment to provide barcoded wristbands for all patients according to UF Health Shands policies;
5. Incident reporting through the UF Health Shands Incident Reporting procedure, in line with the Incident Reporting and Investigation Policy; and
6. Implementing recommended actions arising from investigations of incidents and audits conducted to monitor compliance with this Policy and ensuring there is resultant organizational learning through the divisional structure and more widely across UF Health Shands.

The Patient Blood Management, Blood Safety and Conservation/Utilization Team is responsible for the implementation of the Blood Transfusion Committee's objectives of promoting safe and appropriate transfusion practice, and providing educational materials to all staff involved in the process of ordering blood and blood transfusion.

The Hospital Transfusion Services/Blood Bank is responsible for:

1. Compatibility testing and issuing of blood products;
2. The ordering and management of blood product and component stocks;

(continued):

CP02.100 (4 of 6)

3. Investigating adverse events and reporting them to UF Health Shands Quality and Risk Management, and externally when necessary (e.g. FDA, The Joint Commission);
4. Monitoring requests for products and product utilization;
5. Training and competence of staff involved in laboratory processes; and
6. Regulatory compliance and the maintenance of a quality management system including the traceability of blood components.

The Medical Staff is responsible for:

1. Prescribing blood, blood components or blood products appropriate to the needs of the patient;
2. Clearly indicating the reason for transfusion, quantity of products and communicating the degree of urgency; and
3. Explaining to patients the risks, benefits and possible alternatives to blood transfusion and providing written information where appropriate, and obtaining and documenting consent or refusal.

Nursing and other Transfusing Staff are responsible for:

1. Requesting delivery or transportation of blood products;
2. Obtaining red cells for transfusion via the electronic remote issue system, ensuring that the right blood unit is correctly labeled for the intended patient and that blood for only one patient is collected at each visit;
3. Carrying out pre-transfusion checks to ensure the right blood is transfused;
4. Monitoring the patient during transfusion;
5. Notification and inclusion of medical staff in the management of the patient if a transfusion reaction should occur;
6. Ordering transfusion reaction investigation and reporting of transfusion reactions or other incidents to the Blood Bank when possible reaction symptoms are observed; and
7. Documentation of transfusion, number of units administered and observations recorded in patients' medical records and for maintaining and updating their training, knowledge, competence and practice.

Phlebotomists and others taking blood samples are responsible for:

(continued):

CP02.100 (5 of 6)

1. Checking the identity of a patient before taking any blood samples;
2. Verifying that information on the request is complete;
3. Using safe techniques for obtaining blood samples;
4. Correct labeling of blood sample tubes in accordance with UF Health Shands procedures; and
5. Reporting incidents when an adverse event occurs.

Blood Transport Staff and others transporting blood components are responsible for:

1. Transporting blood components, using a pick-up slip or patient label obtained from the clinical area, and verifying the patient's info on pick up slips or patient label;
2. Matching blood components and compatibility tag to ensure that the blood component being transported is correct for the intended recipient;
3. Only transporting blood components for one patient at a time; and
4. Informing the Hospital Blood Transfusion Laboratory immediately of any discrepancies between a pick-up slip or patient label and compatibility label attached to a blood component removed from a cooler, pneumatic tube station, or remote blood refrigerator and returning blood components to the Blood Transfusion Laboratory at the request of a clinical area.

All Staff involved in the blood and blood products process are responsible for maintaining and updating their training, knowledge, competence and practice.

CORE PROCEDURE :

1. Blood Transfusion Procedure – [BA-P-004 \(electronic\)](#)
2. [Consent](#) or [Refusal](#) (Transfusion Directive) – [Consent for Treatment](#)
3. Identification of the Patient - [Patient ID](#) & [Blood Bank Specimen](#)
4. Ordering and Prescribing Blood – Ordering [Manual](#) or [Electronic](#) ([flowchart](#))
5. Adult MTP [CP02.008](#) and Pediatric MTP [CP02.018](#)
6. Requesting [pre-transfusion compatibility testing](#)
7. [Requesting blood](#)
8. Preparation of the Patient and Blood Sample Collection – [Patient ID & Specimen Collection](#)
9. Preparing and [dispensing](#) blood components to be transfused

(continued):

CP02.100 (6 of 6)

10. Identifying the patient immediately prior to starting the transfusion ([bedside check](#))
11. How transfusions are administered and documented [electronically](#) and manually
12. Special considerations; Other Blood Components; ABO-incompatible & compatible solid organ [Transplants](#); [Bone Marrow Transplant](#) & Biotherapies; [Research](#) Products; [Blood Substitute](#), Derivatives (Rig), [Management of Transfusion Refusal](#) and Alternatives
13. Recognizing & Managing Transfusion Reaction - [TRX](#)
14. [Reporting Incidents](#)
15. [Risk](#), Audit and Assessment
16. Training and [Blood Administration](#) Skills/Competence Assessment
17. Monitoring Compliance ([TJC](#), [SABM](#), [AABB](#), [FDA](#), [CAP](#), COLA)
18. Transfusion Guidelines – [Adult](#), [Peds](#), [Infant](#), [Adult ECMO](#) & [Peds ECMO](#)
19. Guidelines for Ordering Blood Products, Available Blood Products at [Shands](#)
20. [MSBOS, Blood & Blood Products for Surgical Services](#)
21. [Patient Blood Management](#), Preoperative [Anemia Management](#) and Alternatives to Transfusion

ASSOCIATED POLICIES:

- See links above

KEY WORDS: Reaction, Anemia, Blood Substitute